INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Bevacizumab 7.5 · lung

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00214
Version 8

Bevacizumab is given by drip on Day 1 every 21 days. In this lung setting it starts with platinum chemotherapy for up to 6 cycles, then may continue alone while helping and tolerable. Your team must supply the partner chemotherapy schedule.

Treatment stages

01 **With chemotherapy**
With chemotherapy

02 **Possible continuation**
Possible continuation

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology team immediately for coughing blood, significant bleeding, severe tummy pain, sudden breathlessness or chest pain, or a new severe headache, confusion, seizure or vision change. Fever or sudden illness also needs urgent advice, especially with chemotherapy.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Three medicines. Different jobs.

To help control advanced non-squamous lung cancer alongside the prescribed chemotherapy, then possibly as continuation treatment.

ANTI-VEGF ANTIBODY

Bevacizumab

Bevacizumab



Blocks the VEGF growth signal

Bevacizumab binds VEGF, a signal that helps new blood vessels grow. It can slow tumour growth but also affect normal blood vessels and wound healing.

Blocks the VEGF growth signal → Cancer growth disrupted → Healthy tissues may also be affected

What to watch for

Report bleeding, severe tummy pain, unusual headache or vision changes urgently. Tell the team about any surgery or wound.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding the evidence

AVAiL showed longer progression-free survival with bevacizumab added to the studied chemotherapy, without a significant overall-survival improvement.

Direct evidence for 7.5 mg/kg with the studied cisplatin/gemcitabine backbone; outcomes must not be borrowed from 15 mg/kg carboplatin/paclitaxel trials or colorectal studies.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment → review → next steps

Treatment stages

01 **With chemotherapy**
With chemotherapy

02 **Possible continuation**
Possible continuation

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

WITH CHEMOTHERAPY
With chemotherapy

1–6

Up to 6 three-week cycles with the named platinum regimen. Bring its schedule and safety plan too.

POSSIBLE CONTINUATION
Possible continuation

Review

Bevacizumab alone every 3 weeks if the team confirms ongoing benefit and acceptable side effects. No fixed total number of cycles.

Your team confirms each stage and discusses any changes with you.

BASELINE → TREATMENT → FOLLOW-UP

My treatment and safety checks

Routine safety tests are separate from scans and tumour markers, which follow your individual plan. No universal imaging or tumour-marker interval is specified.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00214 v8 · official regimen · Avastin current EMA product information

THE WHOLE TREATMENT JOURNEY

Bevacizumab 7.5 · lung: treatment and checks

First-line non-squamous non-small-cell lung cancer (NSCLC) · bevacizumab component only

Bevacizumab is given by drip on Day 1 every 21 days. In this lung setting it starts with platinum chemotherapy for up to 6 cycles, then may continue alone while helping and tolerable. Your team must supply the partner chemotherapy schedule.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment NCCP 00214 · SMPC	Confirm lung indication / partner	Bevacizumab Day 1	Follow partner schedule	Up to 6 combined cycles; review	Continuation only if agreed
Bloods / urine NCCP 00214 · SMPC	full blood count (FBC), kidney/liver, urine protein	Regular blood/urine checks	Quantify protein if needed	Review trends	Follow abnormalities
Blood pressure / heart NCCP 00214 · SMPC	blood pressure (BP) and cardiac history/exam	BP before each cycle	Extra BP if raised	Review control	BP post-treatment
Scans / other tests NCCP 00214 · SMPC	Individual disease plan	No fixed scan interval	international normalised ratio (blood-clotting test) (INR) if warfarin	Response review	Follow-up agreed

What each check is for

1

Prepare

Before treatment

To help control advanced non-squamous lung cancer alongside the prescribed chemotherapy, then possibly as continuation treatment.

Ask / act: Confirm the aim, phase, prescription, tests and contacts.

2

Review

Before each cycle / at agreed reviews

Check benefit and tolerability

Ask / act: Report symptoms and confirm the next plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Partner chemotherapy

This covers bevacizumab only. Bring the separate chemotherapy schedule, anti-sickness plan and urgent-contact instructions.



STEP 2

Blood pressure and urine

These checks can find problems before you feel unwell. Take blood-pressure medicines as prescribed; do not adjust them yourself.

STEP 3

Surgery and dental procedures

Tell oncology before any operation or invasive procedure. The team must agree treatment pauses and assess wound healing.



STEP 4

Support medicines

Bevacizumab alone does not usually need routine anti-sickness prevention. Chemotherapy partners may need a different plan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Bevacizumab infusion	 **Day 1** Day 1 every 21 days after review.
Partner chemotherapy plan	 **Days 1–21** See the named chemotherapy schedule; additional treatment visits may occur.
Symptom awareness	 **Days 1–21** Bleeding, headache, tummy pain and new symptoms can occur at any time.

Symptoms can occur at any time: Side effects and infection do not follow a guaranteed day-by-day risk curve. Contact the team when symptoms occur.

Changes for safety: Treatment may be delayed, reduced or stopped. Do not make up missed doses or change your tablets yourself.

> **Why these bands stay broad:** Relative days without personal dates. The team confirms actual treatment and test dates. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

> **There are no guaranteed "safe days".** Call your oncology team immediately for coughing blood, significant bleeding, severe tummy pain, sudden breathlessness or chest pain, or a new severe headache, confusion, seizure or vision change. Fever or sudden illness also needs urgent advice, especially with chemotherapy.

Between-treatment notes

Day 1: Bevacizumab infusion

After blood pressure, urine and other safety checks. Partner chemotherapy follows its own schedule. Use the current prescription and contact plan.

Day 2–21: Between bevacizumab doses

Watch symptoms, take only prescribed support medicines and follow the separate chemotherapy plan. Use the current prescription and contact plan.

Support around the medicines

1  **Blood pressure and urine**

2  **Support medicines**

Questions worth asking

Who confirms my next dose?

Your treating team checks the prescription, tests and symptoms. A completed-cycle stamp records past treatment; it does not authorise the next dose.

What if the plan changes?

Ask the team to update your phase, medicines, test appointments and cycle record. Use the updated prescription.

When are my scans?

Your team sets a response and follow-up plan for your cancer and symptoms. This document does not invent a fixed scan or tumour-marker timetable.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Bevacizumab component · combination or continuation

Active medicines: Bevacizumab

Emetogenic risk: Minimal; partner systemic anticancer therapy (SACT) separate

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis for bevacizumab alone; select as-needed support under local policy.
- Do not use this component plan as the antiemetic plan for platinum chemotherapy.
- Premedication is not usually required unless a previous hypersensitivity warrants an individual plan.
- Blood-pressure treatment, urine-protein follow-up and procedure/wound timing need an individual plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Head / blood pressure

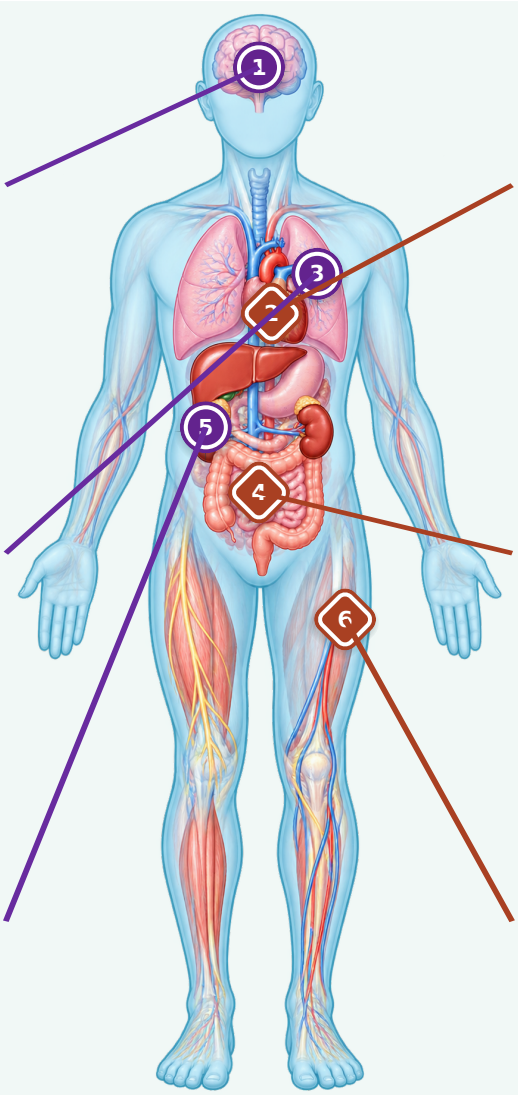
Bevacizumab

New severe headache, confusion or visual change needs urgent advice.

2 Heart / blood vessels

Bevacizumab

Chest pain, sudden breathlessness or a swollen painful leg needs urgent help.



3 Lung bleeding

Bevacizumab

Coughing up blood needs immediate assessment.

4 Bowel / abdomen

Bevacizumab

Severe tummy pain or a rigid swollen abdomen needs urgent assessment.

5 Urine / kidneys

Bevacizumab

Urine tests detect protein; report swelling or much less urine.

6 Bleeding / wounds

Bevacizumab

Report bleeding and unhealed wounds; tell the team before procedures.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call your oncology team immediately for coughing blood, significant bleeding, severe tummy pain, sudden breathlessness or chest pain, or a new severe headache, confusion, seizure or vision change. Fever or sudden illness also needs urgent advice, especially with chemotherapy.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00214 v8 · official regimen**
   https://healthservice.hse.ie/documents/6554/214_Bevacizumab_7.5.pdf
2. **Avastin current EMA product information**
   https://www.ema.europa.eu/en/documents/product-information/avastin-epar-product-information_en.pdf
3. **NCCP 00214 v 8 and current antiemetic classification**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
4. **NCIS medical oncology antiemetic medicines**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **AVAiL final analysis**
   https://pubmed.ncbi.nlm.nih.gov/20150572/